Omeprazole for Leo Martins

Order ord_ed3e6940 · AWAITING_REVIEW · prescribed by Dr. Ada Okafor

Rx: 8 mg, 1×/day, 30 days, oral | 0.75 y · 8.5 kg | State Medicaid (demo)

Conditions

- Gastroesophageal reflux
- Young child (< 6 years)

Allergies / avoids

None.

Current medicines

None.

Proposed formulation

Strength	Make	Beyond-use date
2 mg/mL	125 mL	30 days

Oral suspension in **Sodium bicarbonate 8.4% vehicle** · 8 mg = 4 mL once daily

Label: Give 4 mL by mouth once daily. Shake well before use. Keep in the refrigerator.

Beyond-use date source: Published stability study, 2 mg/mL in 8.4% sodium bicarbonate

Insurance coverage

State Medicaid (demo) · member MCD-55-019 · synthetic demo plan

Recommended	Patient copay	Prior authorization
compounded Omeprazole 2 mg/mL oral suspension	$1	Not needed

Covered options

- compounded Omeprazole 2 mg/mL oral suspension: $1 copay
- Omeprazole oral suspension: $3 copay, prior auth (Prilosec (Covis Pharma US, Inc))

Denied or unsuitable

None.

Message to prescriber

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Dr. Ada Okafor: no change needed for Leo Martins. We will prepare compounded Omeprazole 2 mg/mL oral suspension.
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Compounding procedure

Ingredients

- 13 × Omeprazole 20 mg delayed-release capsules (260 mg; 250 mg needed)
- Sodium bicarbonate 8.4% vehicle q.s. to 125 mL

Equipment

- Mortar and pestle
- Graduated cylinder
- Amber plastic bottle
- Oral syringes

Steps

1. Calculate: 2 mg/mL × 125 mL = 250 mg Omeprazole.
2. Gather 13 × Omeprazole 20 mg delayed-release capsules (260 mg; 250 mg needed).
3. Open the capsules and empty the pellets into the mortar.
4. Add a small amount of Sodium bicarbonate 8.4% vehicle and levigate into a smooth, lump-free paste.
5. Add more vehicle in portions (geometric dilution), mixing well after each addition.
6. Transfer to a calibrated amber bottle, rinse the mortar with vehicle, add the rinse, then add vehicle to a final volume of 125 mL (q.s.).
7. Cap and shake well for 1 minute.
8. Label and store: refrigerate.

Quality checks

- Uniform appearance, no clumps
- Final volume checked
- Re-disperses easily on shaking
- Second person verifies calculation

Label text

- Give 4 mL by mouth once daily. Shake well before use. Keep in the refrigerator.
- Discard 30 days after the compounding date (beyond-use date).
- Compounded medication. Keep out of reach of children.

Demo only. A licensed pharmacist must verify every step.

Safety findings

WARN · regulatory — 1 commercial product(s) look suitable for this patient: Prilosec (Covis Pharma US, Inc, oral suspension). Under FDA section 503A, document the clinical reason a compounded version is needed.

`openFDA drug label set_id b6761f84-53ac-4745-a8c8-1e5427d7e179`

INFO · dose — Dose is within the pediatric range.

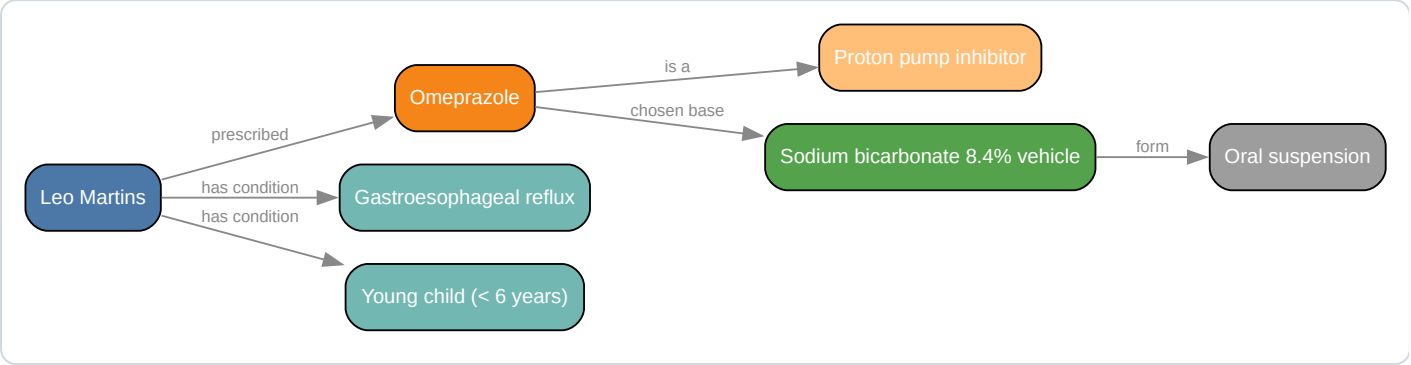
`8 mg × 1/day = 8 mg/day`

`8 mg ÷ 8.5 kg = 0.94 mg/kg/day`

INFO · coverage — Best covered option on State Medicaid (demo): compounded Omeprazole 2 mg/mL oral suspension at $1 copay, no prior auth.

Evidence graph

What the agents looked at in FalkorDB.



Patient Drug DrugClass Base Excluded base Excipient ExcipientGroup Condition DosageForm

Agent trace

Intake · *validate_prescription*
Omeprazole 8 mg, 1x/day for 30 days (oral) for Leo Martins.

History · *derive_facts*
Age 0.75 y: added derived condition(s) young_child.

History · *load_profile*
Leo Martins, 0.75 y, 8.5 kg. Conditions: Gastroesophageal reflux, Young child (< 6 years). Allergies: none. Current meds: none.

Formulation · *choose_form*
Patient needs a liquid because of: Young child (< 6 years).

Formulation · *screen_bases*
1 candidate base(s); 0 excluded.

Formulation · *propose_formulation*
Omeprazole 2 mg/mL oral suspension in Sodium bicarbonate 8.4% vehicle (stability study, BUD 30 days).

8 mg = 4 mL once daily. Make 125 mL.

Safety · *safety_review*
0 blocking, 1 warning, 1 info finding(s).

Coverage · *check_coverage*
State Medicaid (demo): 2 covered option(s), 0 denied or unsuitable. Recommend: compounded Omeprazole 2 mg/mL oral suspension.

Generated by Compounder (FalkorDB + agents). Demo only, not for clinical use. All patients and insurance plans are synthetic.